SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Pemetrexed + Cisplatin

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Pleural mesothelioma / non-squamous NSCLC · chemotherapy component · NCCP 00317 · version 7 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Pemetrexed + Cisplatin is chemotherapy used for selected people with unresectable pleural mesothelioma or non-squamous non-small-cell lung cancer. The aim may be disease control or, within a separate combined preoperative plan, support for surgery. Ask your team to explain the expected benefit for your own diagnosis.

Schedule
Both chemotherapy medicines are planned on Day 1 of a 21-day cycle. For pleural mesothelioma, the source describes up to 4–6 cycles. For advanced non-squamous NSCLC, it describes 4–6 combination cycles followed, when appropriate, by a separate pemetrexed maintenance plan (NCCP00222). The selected preoperative nivolumab combination uses three cycles and a separate immunotherapy plan (NCCP00849). Your team confirms which course applies and may stop or change it earlier.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Pemetrexed
ANTIFOLATE CHEMOTHERAPY

- ☐ Low blood counts, infection and bleeding
- ☐ Nausea, vomiting and fatigue
- ☐ Mouth soreness and diarrhoea
- ☐ Kidney problems
- ☐ Allergic or infusion reactions
- ☐ Skin rash; rarely serious skin reactions
- ☐ Rare serious lung inflammation

Cisplatin
PLATINUM CHEMOTHERAPY

- ☐ Low blood counts, infection and bleeding
- ☐ Nausea, vomiting and fatigue
- ☐ Mouth soreness and diarrhoea
- ☐ Kidney problems
- ☐ Allergic or infusion reactions
- ☐ Hearing changes or tinnitus, which can persist
- ☐ Numbness or tingling, which can persist
- ☐ Electrolyte changes and need for hydration

Confirm the treatment aim and indication, likely benefit and uncertainty, reasonable alternatives including not proceeding, serious and lasting risks, kidney/hearing/nerve problems, fertility and pregnancy advice, vitamin and steroid plans, and the chance of delays or dose changes.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00317 v7.

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Developed by Dr Paul O'Brien